BASELINE → TREATMENT → FOLLOW-UP

Treatment safety, neuroendocrine tumour (NET) response and follow-up

Your team books and reviews each test. Response scans and hormone or tumour markers follow an individual plan; they are not automatically required at every medicine review.

Check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver	Baseline tests	Before each 28-day cycle	Earlier tests if symptoms or dehydration	Review results before prescribing	Follow persistent abnormalities
Blood pressure / heart	Baseline blood pressure (BP); cardiac assessment and echo/ multigated acquisition (heart pumping) scan (MUGA) if indicated	BP at every attendance; heart tests if indicated	Report chest pain, breathlessness or swelling	Agree management of BP or heart changes	Individual follow-up
Thyroid / urine / glucose	Thyroid tests; baseline urine assessment; glucose review	Thyroid every 12 weeks; urine/ glucose as clinically indicated	Report cold intolerance, marked fatigue or low-glucose symptoms	Review medicine effects and diabetes treatment	Follow unresolved changes
NET response	Record symptoms, disease and personal goals	Consultant-directed imaging / markers	Track symptoms and adherence	Assess benefit and treatment burden	Plan next treatment or surveillance

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00327 v5 · current regimen · EMA product information and patient leaflet

Tests in your plan

5-hydroxyindoleacetic acid (5-HIAA)

If part of your plan

WHEN

Only if requested for your hormone syndrome; the team sets the dates.

Measures a breakdown product of serotonin to help assess selected neuroendocrine tumours.

Often a timed urine collection. Follow the laboratory’s food and medicine instructions.

Chromogranin A

If part of your plan

WHEN

Only when the team considers it informative for your tumour.

A selected tumour-marker result, interpreted with symptoms and scans.

Acid-suppressing medicines and other conditions can alter the result. It is not a routine test before every treatment; do not stop medicines yourself.

Selected pancreatic hormones

If part of your plan

WHEN

Only for the hormone question identified by your specialist.

Examples include gastrin, insulin or glucagon when the diagnosis or symptoms call for them.

The team names the test, preparation and review date; these are not a standard panel for every patient.

THE WHOLE TREATMENT JOURNEY

Sunitinib 37.5 mg · pancreatic NET: treatment and checks

Progressive unresectable or metastatic well-differentiated pancreatic neuroendocrine tumour under NCCP 00327a.

Take the prescribed sunitinib dose once daily continuously, at the same time, consistently with or without food. The 28-day cycle is a review period, with no planned capsule-free week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver NCCP 00327	Baseline tests	Before each 28-day cycle	Earlier tests if symptoms or dehydration	Review results before prescribing	Follow persistent abnormalities

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
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NET response NCCP 00327	Record symptoms, disease and personal goals	Consultant-directed imaging / markers	Track symptoms and adherence	Assess benefit and treatment burden	Plan next treatment or surveillance

What each check is for

1 Continuous daily sunitinib

28-day review

Sunitinib is a daily targeted capsule used to help control a pancreatic NET that cannot be removed or has spread and is progressing. The team reviews control, side effects and your priorities.

Ask / act: Confirm the current prescription, tests and support medicines.

Sources: NCCP 00327

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.37